THE MATERNITY METHOD

NCLEX CHEAT SHEET

Antepartum Assessment & Discomforts of Pregnancy

Maternal & Newborn › Antepartum

The Big Picture

Prenatal care **checks on mom and baby at every visit**, finds problems early, and **teaches** what's normal vs. what's a warning sign. Most discomforts come from **hormones** (progesterone, hCG, relaxin) and a **growing uterus** pressing on everything around it.

KNOW THE NUMBERS

LMP − 3 mo + 7 d
Naegele's rule for due date (add 1 yr)

20 wks
Fundus at the **umbilicus**

24–28 wks
Glucose screen

28 wks
RhoGAM if mom is Rh−

36–37 wks
GBS swab

Signs of Pregnancy — Who notices it?

Type	Examples	Why it matters
Presumptive: the patient notices	Missed period, nausea, breast tenderness, fatigue, peeing more, **quickening** (first flutters, about 16–20 wks)	Could be caused by something else. **Least** reliable.
Probable: the provider finds it	**Goodell** (soft cervix), **Chadwick** (bluish cervix/vagina), **Hegar** (soft lower uterus), ballottement, Braxton Hicks, positive pregnancy test	Strongly suggests pregnancy but still **not proof**
Positive: proof of a baby	Fetal heart tones heard, baby seen on **ultrasound**, fetal movement **felt by the examiner**	Only these **confirm** pregnancy

MEMORY HACK · GTPAL

Count pregnancies, not babies. Twins = 1 G, 1 birth, 2 L

G
Gravida: all pregnancies, including this one

T
Term births at 37+ wks

P
Preterm births at 20–36 6/7 wks

A
Abortions: losses before 20 wks (any cause)

L
Living children now

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Fundal Height & Visits

- **12 wks:** just above the pubic bone
- **20 wks:** at the umbilicus
- **20–36 wks:** height in cm ≈ weeks (±2 cm)
- **36 wks:** near the xiphoid; drops with **lightening**
- **Visits:** every 4 wks until 28, every 2 wks until 36, then **weekly**

Measuring **bigger or smaller** than dates? Think twins, extra fluid, wrong dates, or a baby not growing well.

Key Screening Timeline

- **First visit:** blood type + **Rh**, antibody screen, CBC, rubella, HIV, hep B, syphilis, urine culture
- **10+ wks:** cell-free DNA; **11–14 wks:** nuchal translucency US
- **15–22 wks:** AFP / quad screen: **high AFP → neural tube defect**, low → Down syndrome
- **18–22 wks:** anatomy ultrasound
- **24–28 wks:** glucose · **36–37 wks:** GBS

Common Discomforts What to teach

Discomfort	Why it happens	Relief tips
Nausea & vomiting (1st trimester)	High **hCG**	Dry crackers **before getting up**, small frequent meals, don't let stomach get empty, ginger, vitamin B6
Heartburn	**Progesterone** relaxes the stomach valve; uterus pushes up	Small meals, **stay upright** after eating, avoid spicy/fatty foods
Constipation & hemorrhoids	Progesterone **slows the gut**; iron pills	Fluids, **fiber**, walking; sitz baths and witch hazel for hemorrhoids
Back pain	Posture shifts forward; **relaxin** loosens joints	**Pelvic tilts**, good posture, low heels, bend with the knees
Leg cramps	Calcium/phosphorus imbalance, pressure on nerves	**Pull toes toward the head** (dorsiflex), calcium-rich foods
Swollen feet & varicose veins	Uterus presses on the big veins	**Elevate legs**, don't cross legs or stand long, compression stockings, rest on the **left side**
Dizzy when lying flat	**Supine hypotension**: uterus squeezes the vena cava	Turn to the **left side** or tilt with a wedge
Sharp groin pain with movement	**Round ligament** stretching	Move slowly; bring knees toward the belly

DANGER SIGNS · TEACH: CALL RIGHT AWAY

Vaginal bleeding or **fluid leaking** · **Severe headache**, vision changes, or upper belly pain · Sudden **swelling** of face or hands · **Baby moving less** (fewer than 10 kicks in 2 hrs) · **Fever**, burning with urination · Vomiting that won't stop · **Contractions before 37 weeks**

☆ MUST-KNOW FOR THE NCLEX

- Only **positive** signs (heart tones, ultrasound, examiner feels movement) confirm pregnancy.
- **GTPAL:** twins count as **one** pregnancy and **one** birth but **two** living children.
- Dizzy and pale while lying flat? Turn her to the **left side** first.
- **High AFP** points toward a neural tube defect; folic acid helps prevent it.
- Fewer than **10 movements in 2 hours** needs to be checked the same day.

QUICK CHECK · TEST YOURSELF

1. LMP was May 10. Using Naegele's rule, the due date is _____.
2. Pregnant now; had twins at 35 weeks (both living) and a miscarriage at 10 weeks. Her GTPAL is _____.
3. A soft lower uterine segment (Hegar sign) is a _____ sign of pregnancy.
4. Fundal height at the umbilicus = about ___ weeks.
5. For a leg cramp, teach her to _____ the foot.

Answers: 1. February 17 · 2. G3 T0 P1 A1 L2 · 3. probable · 4. 20 · 5. dorsiflex (pull toes toward the head)

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